Resolution

World Health Organisation

February 15, 2026

Committee: World Health Organisation (WHO)
Topic: The Question of Addressing Global Health Standards in Vulnerable and Underdeveloped Regions
Main Submitters: The Islamic Emirate of Afghanistan, The Federal Republic of Germany
Co-submitters: South Sudan, Syrian Arab Republic
Signatories: Somalia, Chad, Brazil, Uganda, Japan, France, Austria, Australia

Advocates for accessible healthcare facilities in all regions with an emphasis on struggling nations,

Addresses healthcare inequalities faced by citizens in third world countries,

Urges long term health system strengthening and humanitarian relief,

Demands a fundamental restructuring of global health governance to prioritise funding and resources,

Recognises its responsibility under the Charter of the United Nations regarding global health standards.

1. **Calls for** a rigid definition of vulnerability in the context of healthcare systems;
    a. Designates 4 main levels of vulnerability;
        i. A level 1 indicates at (low) risk of having healthcare interrupted by an external factor including but not limited to; international conflict, natural hazards, and suspensions in trade
        ii. A level 2 indicates has an inconsistent healthcare system with a lack of high-quality care;
        iii. A level 3 indicates has a dangerous healthcare system with a lack of highly trained professionals and resources;
        iv. A level 4 indicates under active attack from a national or international force which warrants immediate referral to the UNSC under the "Responsibility to Protect" (R2P) protocols;
    b. Understands that certain regions may not fit into a specific level of vulnerability, and allows for means to request particular considerations by the WHO for a specialized action plan

2. **Demands** the recognition for categorized patient risk levels worldwide;
    a. Designates 6 main levels of risk;
        i. Level 1 is categorized by illness which can be treated virtually or via a local healthcare worker
        ii. Level 2 is categorized by prolonged illness impacting everyday life and preventing the enacting of daily tasks

iii. Level 3 is categorised by severe illness requiring hospitalization meaning they can request for a referral to another nearby healthcare facility capable of more effective treatment
iv. Level 4 is categorized by critically ill patients with a possibility of death and may be eligible for possible health based migration to a neighbouring or nearby partner country
v. Level 5 is categorized by a patient who is actively dying and capable of recovery with high quality care and may request for relocation to a capable healthcare facility (in a developed country)
vi. Level 6 is categorized by disabled patients with no ability to survive under current circumstances which warrants health mandated permanent migration for a quality life

3. **Demands** the standardization for vulnerable regions of rates for life expectancy, mortality, and disease prevalence;
    a. Aims for 80 years of age as a global life expectancy worldwide
    b. Aims for less than 70 maternal deaths per 100,000 females worldwide
    c. Aims for less that 12 infant deaths per 1,000 births worldwide
    d. Aims for less than 25 deaths of children under 5 per 1,000 worldwide
    e. Aims for a reduction of global cause specific mortality rates by 33.3%
    f. Aims for a reduction of global tuberculosis prevalence rates by 80%
    g. Aims for a reduction of global malaria prevalence rates by 90%

4. **Urges** increased international collaboration and funding to support less developed nations in removing barriers to healthcare education and awareness;
    a. Utilising flexible curriculums to educate citizens in rural, hard-to-reach areas;
    b. Supporting local teacher/medical professional training programs;
    c. Developing monitoring mechanisms within schools to measure healthcare awareness and understanding;
    d. Invites a collaboration of qualified healthcare professionals to pass knowledge internationally onto intermediate medical practitioners for the benefit of vulnerable nations;
        i. Provides a proportionate wage supported by the Global Fund.
        ii. Provides free visas for qualified healthcare professionals in developed regions for the migration to less developed countries to aid in strengthening their systems.
    e. Forming partnerships between undeveloped and developed countries for the purpose of hosting health mandated migration for critical patients;
    f. Distributes supplies, staff, and sustainable infrastructure sufficiently between each member state;
    g. Mandating the abolition of prejudiced educational barriers such as the limitations on secondary education for women in undeveloped nations.

5. **Requests** the creation of specialised Village Supply Kits which shall be distributed bimonthly to underdeveloped and vulnerable regions;
    a. Takes note of specific village needs, supplying materials which include but are not limited to;
        i. Any required essential medicines;

ii. Blankets and non-perishable food items;
iii. Baby food & formula;
iv. Feminine hygiene products (period underwear for reusability);
v. Clean modesty covering;
vi. Bandages & medical tape;
vii. All essential vaccines;
viii. At least one telemedicine device per 1,000 people;

b. Monitoring connected via non-terrestrial internet (Satellite);
c. Calls for the creation of a specialised task force for the purpose of protecting supply chains and distributors;
d. Authorises the publication of transparent monthly supply, situation, and usage reports;
e. Emphasizes the fact that specific necessities may change.

6. **Establish** a temporary peacekeeping authority to prevent destructive authorities from influencing the access to basic human rights. The authority would;
   a. Address any violations of human rights under the authorities control;
   b. Appoint members of the conflict nation's community to manage any governmental decisions while transitioning those qualified individuals into the official national authorities;
   c. Ensure neutrality in healthcare facilities to protect the care and peace of all patients;
   d. Assist those who are injured, infirmed, ill, disabled, or in need of urgent medical care;
   e. Enable mechanisms to fund healthcare such as a good distribution of tax;

7. **Reaffirms** an specialized national applicable plan for the time of emergencies, to ensure the safety of the citizens of the region;
   a. Endorses the WHO Humanitarian Response Plans;
   b. Developing evacuation routes for tsunamis in remote coastal regions;
   c. Appoint protocols for the occurrence of earthquakes;
   d. Establishing reparation methods for floods;
      i. Government relief funds;
      ii. Disease surveillance
      iii. Improving water, sanitation, and hygiene services;
   e. Urges the creation of water reservoirs to protect the nation against droughts and food and water shortages;
      i. Preventing waterborne and respiratory diseases through adequate supply of water and Clause 8's health training;
   f. Expresses its hope to refortify old buildings with modern impact-resistant glass, and impact-resistant storm shutters while adhering to ISO guidelines;

8. **Further requests** the establishment of community-based institutional care and the use of AI and digital health services for mental health and psychosocial systems;
   a. Advocates for cooperation with local institutions to promote mental health-care in a way that does not further emphasize stigma and adaptable for the local citizens,
   b. Emphasizes a secure multilayered response through the use of family/community support;
   c. Calls for a response that is aligned with operational guidance on MHPSS and Inter-Agency Standing Committee (IASC) guidelines;

d. Expanding the MHPSS Pyramid Approach to encompass all vulnerable and developed regions;
    i. Specialized services;
    ii. Focused, nonspecialized services;
    iii. Strengthening community and family support;
    iv. Social consideration in basic services and security;
e. Arranges commodities and services to make mental health care more accessible and convenient for people in underdeveloped regions,

9. **Requires** the training of local civilians alongside the provision of village health kits and mandates a minimum of training in;
    a. Basic sutures and stitches (provide fake skin for practice);
    b. Life saving techniques (CPR);
    c. Common disease symptom;
    d. Solutions to malnutrition;
    e. Chronic illness & disability care;
    f. Pregnancy risks & birthing procedures;
    g. Prenatal, neonatal, and antenatal care;
    h. Basics of human development;

10. **Proposes** collaboration between various UN committees for the most substantial outcome;
    a. Requests financial aid and policy guidance from the Economic and Financial Committee (ECOFIN) in improving the economy of underdeveloped regions, in order to ensure that citizens of the member states can afford basic healthcare and better living qualities;
    b. Urges local institutions to adopt Food and Agriculture Organization (FAO)'s practices, to guarantee that the people receive nutritious food, for better health in the long run;
    c. Affirms aid from the United Nations High Commissioner on Refugees (UNHCR) to safely and consensually replace displaced individuals that have become victims of political or natural disasters;
    d. Collaborates with the United Nations Human Rights Council (UNHCR) to secure fair rights for citizens in vulnerable regions with an emphasis on protecting internally displaced individuals ineligible for refugee aid.

Ideas

- Incentivize highly trained professionals from developed countries to migrate to less developed nations to support their healthcare systems
    - Free visas
    - Wage and housing supported by a Global Health Fund
- Village health kits (supplied every 2 months)
    - Supplies include;
        - Any required essential medicines
        - Blankets and non-perishable food items
            - Baby food & formula
            - Feminine hygiene products —> period underwear for reusability
            - Clean modesty covering
        - Bandages & medical tape
        - At least one telemedicine device per 1,000 people
            - Connected via non-terrestrial internet (Satellite)
        - Portable washing machine or device
- Local training programs to prevent the need for hospitals in non dire situations
    - Training on basic stitches (provide fake skin for practice)
    - Training on life saving techniques (CPR)
    - Training on signs of dying
    - Training on chronic illness & disability care
    - Training on pregnancy risks & birthing
    - Training on prenatal, neonatal, and antenatal care
    - Training on basic human development
- Standards customized to the circumstances
- Defining vulnerable
    - Level 1 : at risk of having healthcare interrupted by an external factor (such as (inter)national conflict, weather, or a suspension in trade)
    - Level 2 : has an inconsistent healthcare system with a lack of high-quality care
    - Level 3 : has a dangerous healthcare system with a lack of highly trained professionals and a lack of resources
    - Level 4 : under active attack from a national or international force → immediate referral to the UNSC under the "Responsibility to Protect" protocols
- Resolve the ban by the Taliban on women's (secondary and higher) education – specifically healthcare training
- Provide minimum standards for essential medications available per person – resolve inequities by establishing global partnerships between high efficacy healthcare systems and at risk systems.
- Offer a deal to the Taliban, if participating in any forum of the UN or an NGO, they must uphold all national and international laws, in the case that this is not immediately upheld – weekly migration groups supported by UN volunteers will assist in safely relocating high risk civilians (including high risk patients) in zones particularly vulnerable to the Taliban to partner countries (determined during the committee)
- A UN facilitated transition authority (made up of several different national volunteers) for the purpose of resolving conflicts in Afghanistan
    - Temporary international residence of multiple nations to mediate conflict and take over from the Taliban and appoint new and trustworthy national leaders with democracy determining the style of government and necessary positions of power
- Free visa program for all Afghan citizens in dire health (level 3+) → partnership with a developed nation (ideally within close proximity)
- Taliban members on duty are not welcome in public healthcare facilities to maintain

the safety and peace of patients → no weapons are permitted
    - Entrances are secured by UN volunteers
- Patient risk levels :
    - Level 1 : can be treated virtually or via a local healthcare worker
    - Level 2 : extended illness impacting everyday life and preventing the enacting of daily tasks
    - Level 3 : severe illness requiring hospitalization —> can request for a referral to another healthcare facility capable of more effective treatment
    - Level 4 : critically ill, possibility of death —> possible health based migration
    - Level 5 : actively dying and capable of recovery with high quality care —> relocation to a capable healthcare facility (in another country if necessary)
    - Level 6 : disabled with no ability to survive under current circumstances —> health mandated permanent migration for a quality life
- All medical facilities must have natural disaster plans regarding possible hazards like floods and earthquakes
- Raise campaigns to prevent corruption
- Mental health infrastructure through digital health care and local training
    - Train locals on basic consultation services and counselling
    - AI chat bots for emotional and mental support

Preamb clauses

Acknowledging
Acting
Affirming
Alarmed by
Alarmed
Anxious
Appreciating
Approving
Aware of
Bearing in mind
Believing
Cognizant
Concerned
Confident
Conscious
Considering
Contemplating
Convinced
Declaring
Deeply concerned
Deeply conscious
Deeply convinced
Deeply disturbed
Deeply regretting
Deploring
Desiring
Determined
Emphasizing
Encouraged
Expecting
Expressing appreciation
Noting with approval
Expressing concern also
Expressing concern
Expressing its appreciation
Expressing its satisfaction
Expressing satisfaction
Firmlyconvinced
Fulfilling
Fully alarmed
Fully aware
Fully believing
Further deploring
Further recalling
Guided by
Having adopted
Having considered
Having considered further
Having devoted attention
Having examined
Having heard
Having received
Having reviewed
Having studied
Having adopted
Having approved
Having considered
Having decided
Keeping in mind
Mindful
Noting
Noting further
Noting with deep concern
Noting with regret
Noting with satisfaction
Observing
Reaffirming
Reaffirming also
Realizing
Recalling
Recalling also
Recognizing
Recognizing also
Recognizing with satisfaction
Referring
Regretting
Reiterating
Reiterating its call for
Reminding
Seeking
Seized
Stressing
Taking into account
Taking into consideration
Taking note
Taking note also
Taking note further
Underlining
Viewing with appreciation
Viewing with apprehension
Welcoming
Welcoming also

Operative clauses

Sample Operative Phrases

Accepts
Affirms
Approves
Authorizes
Calls
Calls upon
Condemns
Confirms
Congratulates
Considers
Declares accordingly
Deplores
Designates
Draws the attention
Emphasizes
Encourages
Endorses
Expresses its appreciation
Expresses its hope
Further invites
Deplores
Designates
Draws the attention
Emphasizes
Encourages
Endorses
Expresses its appreciation
Expresses its hope
Further invites
Further proclaims
Further reminds
Further recommends
Further requests
Further resolves
Has resolved
Notes
Proclaims
Reaffirms
Recommends
Regrets
Reminds
Requests
Solemnly affirms
Strongly condemns
Supports
Takes note of
Transmits
Trusts

Formatting Example

United Nations Security Council

September 20th, 2025

Committee: United Nations Security Council (UNSC)
Topic: Addressing the Rising Threats of Houthi Attacks on International Shipping in the Red Sea
Main Submitter: United States
Co-submitters: United Kingdom
Signatories: United Arab Emirates, The Republic of Korea, Japan, Switzerland

Reaffirms its responsibility under the Charter of the United Nations for the maintenance of international peace and security,

Expresses its concern over the threat that unlawful acts against the safety of navigation pose to seafarers and others,

Underscores the importance of navigational rights and freedom of vessels of all states in the Red Sea, including for vessels transiting through the Baab al-Manadab, in accordance with international law,

Emphasises that the transit passage of merchant and commercial vessels through the Red Sea must continue unimpeded

Recalling its resolution regarding Yemen, and previous attacks against oil terminals under the Yemeni government,

1. **Calls** upon the Houthi armed groups to immediately cease all attacks against commercial vessels and comply with international law, including the protection of civilians, which;
    a. Demands that all armed actions, including missiles, explosives, small arms, or improvised vessels, against commercial shipping in the Red Sea and Gulf of Aden stop immediately;
    b. Requires the immediate release or safe passage of any vessels, crew, or cargo currently held or threatened by Houthi forces;
    c. Insists on full compliance with international humanitarian law, including the protection of civilians and avoidance of targeting non-military personnel or infrastructure;
    d. Requests the Secretary General to establish a monitoring mechanism to verify cessation of hostilities, including reporting incidents to the Security Council.

2. **Encourages** immediate diplomatic negotiations between Houthi representatives, Yemen, and relevant regional stakeholders, with support from the United States and other Security Council members, to achieve a sustainable cessation of hostilities;

a. Requests the appointment of a UN Special Envoy or mediator to coordinate and facilitate direct talks between all parties, ensuring neutrality and transparency;
b. Calls for negotiations to include a clearly defined agenda, covering: cessation of attacks, protection of maritime commerce, humanitarian access, and compliance with international law;
c. Urges participation of regional actors, including Gulf Cooperation Council members and neighboring states, to provide guarantees and confidence-building measures during the negotiation process;
d. Encourages the establishment of a timeline and benchmarks for progress with regular reports to the Security Council on compliance outcomes.
e. Directs all member states involved to ensure the protection of all commercial and shipping vessels passing through the Suez Canal, regardless of national affiliation.

3. **Authorizes** the deployment and coordination of international naval patrols in the Red Sea and Gulf of Aden under existing frameworks, with the purpose of deterring and preventing attacks on commercial shipping, which;
    a. Establishes a Joint Maritime Coordination Center to oversee multinational patrol operations and facilitate communication between naval forces;
    b. Directs contributing member states to assign at least one naval vessel or aerial surveillance asset to rotational patrols surrounding the central Red Sea and Gulf of Aden;
    c. Authorises patrol forces to escort vulnerable vessels, including oil tankers, food shipments, and humanitarian aid carriers, through designated safe transit corridors;
    d. Instructs the JMCC to issue weekly updated maps of secure shipping lanes, based on ongoing risk assessments and satellite surveillance;
    e. Tasks the UN Secretary General with submitting a quarterly evaluation of naval patrol effectiveness, including recommendations for adjustments in force levels, patrol routes, and/or rules of engagement;
    f. Authorises naval forces participating in international patrols to use defensive and proportionate force to deter or respond to attacks on commercial vessels, including the interception of armed Houthi boats, while minimising civilian casualties and complying with international law.

4. **Mandates** closer cooperation with the International Maritime Organization (IMO) to establish secure maritime corridors for civilian vessels transiting the Red Sea;
    a. Requests the IMO to develop clear guidelines for vessel routing and reporting procedures to minimize exposure to Houthi attacks;
    b. Encourages the implementation of a vessel identification and tracking system for ships transiting high-risk areas;
    c. Urges member states to provide real-time intelligence and risk assessment to the IMO to assist in corridor planning;
    d. Encourages the prioritization of humanitarian aid vessels and transportation alleys in order to ensure Yemeni security and sovereignty.

5. **Authorises** proportionate and necessary force by member states against Houthi armed groups in response to continued attacks on commercial shipping, should negotiations and diplomatic efforts fail to achieve a full cessation of hostilities;

a. Permitting member states participating in international naval patrols to engage Houthi vessels or forces responsible for attacks, using force that is strictly necessary to neutralize the threat while minimizing civilian casualties;
b. Authorises interception, seizure, or disabling of Houthi maritime assets directly involved in attacks against commercial vessels or naval patrols;
c. Allows use of self-defense measures, including warning shots, disabling fire, and precision strikes against armed Houthi personnel or equipment actively threatening shipping routes;
d. Requires all military action to comply with international law, including the Law of Armed Conflict and rules on proportionality and distinction between combatants and civilians;
e. Encourages coordination among regional and international forces to maximise effectiveness, including shared intelligence, joint patrols, and rapid response to threats.

6. **Establishes** a secure multilateral communication and information sharing network among member states participating in Red Sea security operations, in order to strengthen coordination and rapid response, which;
    a. Mandates the Joint Maritime Coordination Center to serve as the central hub for classified communications and emergency alerts;
    b. Encourages member states to designate national focal points to ensure daily connectivity and immediate exchanges of threat assessments;
    c. Directs the JMCC to provide regular briefings to the Security Council summarizing key intelligence trends, without compromising sensitive operational details.

Main Sub Speeches

Germany: Honorable chairs, esteemed delegates.
It has come to Germany's attention that despite all the aid, all the assistance, all the contributions, and all the donations, to the vulnerable countries,those nations still remain in an unmoving position regarding their healthcare. Healthcare dictates the development of countries; for being wealthy is having physically and mentally healthy citizens. Which is why, Germany PROUDLY presents Resolution 1.2 alongside the The Islamic Emirate of Afghanistan, and other nations that long for a thorough solution to control the inequality undeniable happening within their country. Resolution 1.2 ensures that health is not seen as a privilege, and that all the donations were equally distributed among all the obsolete nations, to finally make progress instead of letting the poor stay poor, and the ill stay untended.

All the contributions made with benevolence are USELESS if the countries that were given finance remain the same as were before BILLIONS of dollars were given.

Thank you, the delegate now yields to Afghanistan.

Afghanistan:

The first 5 clauses demand for recognition, assistance, and accommodation. While the following clauses demand cooperation, progression, and protocols. Resolution 1.2 values and protects not just regular citizens, but every sentient individual living on planet earth. Where in clause 4 ensures recognition from sickly youth to elderly women living in remote villages. This resolution addresses more than the health standards themselves but expands the consideration to political systems, natural hazards, and various societal factors as well as disabilities, chronic illnesses, and mental health issues. The delegation of Afghanistan firmly believes that this resolution is necessary to provide a basis of action plans to combat the insufficiency of the current global healthcare systems. Furthermore, Clause 5 concerns the rights for universal citizens to have access to clean necessities they must consume in their daily lives. Right now, as the committee remains optimistic, an Afghan woman sits to mourn the death of her newly born child because she was unable to access basic sanitation or medical check-ups during her pregnancy. This can and will be rectified by the implementation of this resolution.

For & Against Speeches

South Sudan - For Speech

Honourable chairs and esteemed delegates

The delegate of South Sudan strongly endorses that resolution 1.2 should be passed. In many less developed nations, South Sudan included, healthcare systems are incredibly weak and severely damaged by factors like climate, conflict, and lack of local healthcare facilities within the area. South Sudan strongly believes that this resolution would greatly benefit international standards and other less developed nations as well. It does not only provide temporary stabilisation, but long term initiatives and guidance less developed nations can follow. Through working together to define a globally recognised definition of vulnerability, nations like South Sudan will finally be able to receive proper attention and support. Our resolution emphasises low-cost, local community based initiatives which the nations in this bloc are greatly dependent on.

For Speech

Syria - Against Speech

Somalia - Against Speech

POIs & POCs

Amendment Speeches

Amendment # 4
Amendment Submitter Country: Afghanistan
Amendment Submitter Email: 980165@verso.ac.th

Please paste the amendment below:

1. **Encourages** the expansion of medical and public health education opportunities for students from vulnerable regions through:
    a. Partially or fully funded scholarships funded by developed Member States for degrees in medicine, nursing, and public health:
        i. Prioritising students originating from least-developed and unserved regions;
        ii. Including tuition, accommodation, learning materials and monthly living costs;
        iii. Requiring scholarship recipients to return and serve in their home countries' public healthcare sector for a minimum of three years;
    b. Bilateral agreements ensuring that scholarship recipients:
        i. Receive practical training in advanced healthcare systems;
        ii. Return to their home countries for a minimum service period to strengthen domestic health systems.

ADD/DELETE/REPLACE (please highlight one)
This is incredibly unfair for students with higher potential not coming from undeveloped regions. The chance of this being fairly implemented is low.

The delegate of Afghanistan firmly believes that the prioritization of students in less developed regions is inconsiderate of the future pathways that will be hindered by their backgrounds and is an unsustainable solution. Furthermore, the provision of tuition, accommodations, and living costs is